

PEPFAR reauthorisation hangs in the balance



Since its creation by US President George W Bush in 2003, the US President's Emergency Plan for AIDS Relief (PEPFAR) has been a crucial component of the global HIV response—supporting HIV programmes in low-income countries hit hardest by HIV/AIDS. Over US\$110 billion of investment through PEPFAR has radically reshaped health systems: increasing surveillance capacity, creating large networks of clinical service facilities, and managing drug supply chains. An estimated 20 million people are currently on HIV treatment and 1.5 million have access to pre-exposure prophylaxis thanks to PEPFAR. The programme has helped to prevent an estimated 25 million HIV-related deaths. Many millions more have likely benefited indirectly from the organisation's activities, not least during the COVID-19 pandemic when health systems built and strengthened to combat HIV were instrumental in tackling the new threat. The US Government and US citizens can justifiably be proud of the extraordinary contributions to global health that PEPFAR has made.

Innovations and expansion in surveillance, treatment, and prevention deployed with PEPFAR's support over the past two decades have helped some of the world's hardest hit and least resourced countries make remarkable achievements. Last year, Botswana, one of the original PEPFAR focus countries, announced that it had achieved UNAIDS 95-95-95 targets. In this issue, a study based on data from the PEPFAR-supported Population-based HIV Impact Assessment in Malawi shows that the country is realistically on track to achieve 95-95-95 targets before 2030.

The fourth reauthorisation of PEPFAR is due to happen by September 30, as it has every 5 years since its inception, and President Joe Biden has asked for \$6.8 billion to fund the programme for the next half decade. With ambitious global targets to reduce the burden of HIV/AIDS by 2030, an ability to make long-term plans is essential. The 5-yearly cycle of PEPFAR renewal, which has historically received bipartisan support, provides stability for countries and allows national HIV/AIDS programmes to plan ahead. But for the first time, reauthorisation is under threat.

Chris Smith, a Republican representative from New Jersey who chairs the House Foreign Relations Subcommittee (which oversees PEPFAR), opposes

reauthorisation unless restrictions are included to stop funds going to groups that provide or support abortion services. Although PEPFAR funds are not used to provide abortion, Republicans opposed to the provision of comprehensive sexual and reproductive health services are concerned that some facilities that do provide abortions might benefit from PEPFAR funds. Such restrictions would be akin to the global gag rule, which President Biden rescinded in the first week of his presidency in 2021, so will unlikely receive support from the Democratic Party or President. As a sticking plaster, several Republican politicians are seeking to implement a 1-year renewal with blocks on funds to groups that support abortion in the hope that next year their party will win the presidential election and therefore have more power to determine the direction of PEPFAR.

A failure to reauthorize the programme will not only cast doubt on the future of PEPFAR-supported projects and limit the ability of national programmes to plan ahead, but also will see some time-limited provisions come to an end. These include the requirement that 10% of funds be spent supporting orphans and children affected by HIV. The doubt over reauthorisation will also jeopardise the recently announced Safe Births Healthy Babies initiative: a 2-year, \$40 million effort to accelerate progress to eliminate vertical transmission in countries with high HIV burdens. As highlighted in Correspondence published in *The Lancet* this week, PEPFAR saves children's lives, and failure to reauthorize in full will cost lives. There is no need to spell out the grim irony.

In 2022, John Nkengasong became the first person from the African continent to take the helm of PEPFAR. Now, in the year of its 20th anniversary, Nkengasong should be looking forward to a period of transformation in the way PEPFAR works, working with countries to continue building sustainable programmes with growing self-reliance as they increase their contributions, building on impressive gains already made, and helping the HIV programmes to re-establish themselves after the COVID-19 pandemic. Failure to reauthorize the programme or limiting it to annual cycles will introduce instability and hinder the long-term assurances needed to develop successful programmes. We hope those seeking to derail PEPFAR's operations will listen to reason and reauthorize the programme in full. ■ *The Lancet HIV*

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For more on PEPFAR see <https://www.state.gov/about-us-pepfar/>
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